3.1 Pneumonia*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cough a lot but cannot bring up the phlegm." **Objective** • Thick yellow sputum • Crackles over the right lower lobe • RR 26/min	**ACTUAL** Ineffective airway clearance related to retained thick secretions as evidenced by crackles, an ineffective cough and tachypnea.	• Coughs up secretions effectively; breath sounds better within 48 hours.	• Auscultate the chest every 4 hours. • Unless fluids are restricted, encourage 2–3 L a day to thin the sputum. • Teach controlled coughing and deep breathing. • Give the nebulized bronchodilator or mucolytic as prescribed. • Carry out chest physiotherapy if ordered. • Suction if he cannot clear secretions himself.	• Met when crackles lessen and the cough is productive. • No progress: reassess and inform the physician.
Subjective • "I get short of breath when I talk." **Objective** • SpO_2 89% on room air • Using accessory muscles • Restless	**ACTUAL** Impaired gas exchange related to alveolar consolidation as evidenced by hypoxemia, dyspnea and restlessness.	• SpO_2 of 94% or more within 24 hours.	• Observe SpO_2, respiratory rate and consciousness. • Sit him in high Fowler; when on his side, keep the good lung down. • Give oxygen as prescribed and check that the device works. • Alternate activity and rest. • Follow ABG results as ordered.	• Achieved at SpO_2 95% with less dyspnea.
Subjective • Reports chills **Objective** • Temperature 38.8 °C • WBC 16 × 10^9/L • Aged 72	**RISK** Risk for shock as evidenced by risk factors: infection with potential progression to sepsis.	• BP, heart rate and urine output stay stable.	• Take vital signs 4-hourly, more often if unstable. • Once cultures are taken, give antibiotics without delay. • Chart urine output. • Report systolic BP below 90 mmHg, HR above 120/min or new confusion.	• Outcome reached while vital signs remain stable.

* **References for this care plan:** [17], [20], [35], [37], [50], [106]. Full entries are listed in the References section of the CPMN book.